جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : سهام لطفي صبح ايوب

المحافظة : الشرقيه

الحي / المدينة : العاشر من رمضان

العنوان : العاشر

رقم البطاقة : 28001073400068

رقم التليفون : 01140627866

تاريخ تسجيل الإستمارة : 20-10-2026

العمر(تاريخ الميلاد) : 07-01-1980 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي
التوقيع:(--------------------------)

المقر بما فيه
التوقيع:(--------------------------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام ماموجرام وسونار علي الثديين [10888] **قسم العلاج :** الاورام

التعليق على التشخيص : ---------------------------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** --------------------------- **التوقيع:** د/محمد كيلاني

الاسم: د/ندى حسن **الوظيفة:** --------------------------- **التوقيع:** ندى حسن

الاسم: د/نرمين رمضان **الوظيفة:** --------------------------- **التوقيع:** نرمين رمضان

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة رئيس اللجنة و مدير المستشفي.

جوستاف روسي

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ------------------------ **ا.د/** ------------------------ **د/** ------------------------ **د/** ------------------------

قرار اللجنة

التشخيص النهائي : --

خطة العلاج : --

المستشفي : --- **قسم :** ---

جهة العلاج : --- **نفقات :** ---

ملاحظات : --

رئيس
المجلس الطبي المتخصص

مدير عام
مديرية الشئون الصحية

ختم
شعار الجمهورية

Name: سهام لطفي صبح ايوب
National ID: 28001073400068

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

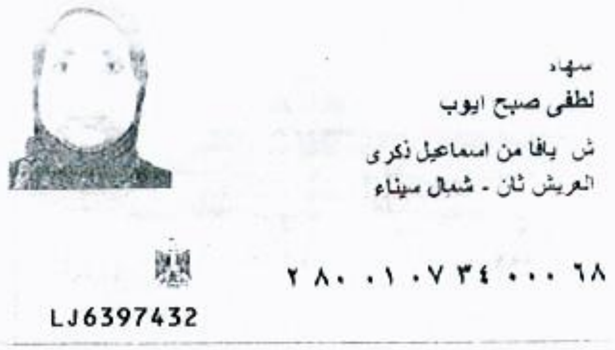
نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : yyyy-mm-dd --:-- --	**تاريخ وساعة الخروج :** 05-07-2026 01:17 PM

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Bilateral mammogram and breast ultrasound**

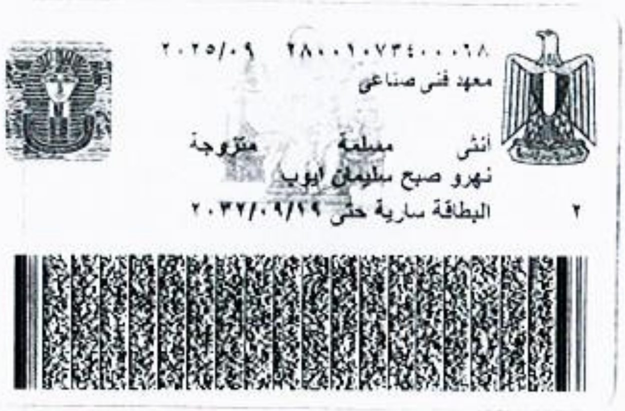
التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين



سهاد
لطفى صبح ايوب
ش يافا من اسماعيل ذكرى
العريش ثان - شمال سيناء

٢ ٨٠ ٠١ ٠٧ ٣٤ ٠٠٠ ٦٨

LJ6397432



٢٨٠٠١٠٧٣٤٠٠٠٦٨ ٢٠٢٥/٠٩
معهد فنى صناعى
أنثى مسلمة متزوجة
نهرو صبح سليمان ايوب
٢ البطاقة سارية حتى ٢٠٣٢/٠٩/٢٩

National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

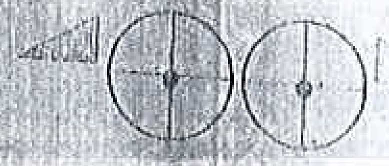
Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

A-Patient ID:
1- Name : سهام لطفى صبح ايوب
Sex: Female
3- Medical number: 03084207
2-ID number: 28001073400068
Old Med : 535/2023
Patient Age : 43 Y

MDT



Family history
marital status
Co morbidity
Phone 01140627865 911
NBS Score
Menopausal status
GROUP
REASON OF MDT POST OPERATIVE
Date of MDT 03/19/2023

Breast Imaging :

Clinical data

Mets Workup :

DX: IDC	**ER:** 7/8	**PR:** 7/8	**Her 2 Neu :** -VE	**SISH :**	**KI67 :**

T	**N**	**M**

Sonomammiography

PATIENT HISTORY

PT FINISHED NACT 4AC+4 TAXOTERE(LAST 29/1/2023) WITH CLINICAL RESPONSE
CESM:COMPLETE RESPONSE
RT SIMPLE MASTECTOMY & RT SLN& RT EXTRA-LNS BIOPSY:
FROZEN SECTION--> FREE SLNS 0/4
COMPLETE PATHOLOGICAL RESPONSE IN PREVIOUSLY ID
FREE NIPPLE & AREOLA
FREE LN 0/9
FREE DEEP MARGIN
STAGE: PT0, PN0

MDT OUTCOME & PLAN

FOR PORT / ZOLADEX/TAMOXIFEN FOR 5 YEARS , shifted to AI / Zoladex
Zometa / 6 month

SIGNATURE

MEDICAL ONCOLOGY			
SURGERY	د/ علاء الاتربى	**RADIOLOGY**	
Psychiatric		**PATHOLOGY**	٢٠٢٣/٠٣/١٩
		Nutrition	

Name: سهام لطفي صبح
MR. : 159

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من بشأن تقرير طبي للمريض

بقسم: رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : تاريخ وساعة الخروج :

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient presented for administration of Zoladex 10.8 mg and was prescribed Femara with supportive therapy for a planned duration of three months.

The patient's pathology demonstrated estrogen receptor (ER) and progesterone receptor (PR) positivity at 7/8, HER2 negativity, and staging of pT0 pN0 M0 as of March 2023. Multidisciplinary discussion recommended a regimen of PORTH, an aromatase inhibitor, and Zoladex. The patient was started on Femara, supportive care, and Zoladex for three months.

The patient received a daycare infusion of Zometa and Zoladex. Laboratory studies performed externally showed serum creatinine of 0.8 mg/dL and ionized calcium of 1.18 mmol/L. The current plan is to continue Zoladex therapy and monitor renal function and calcium levels.

تحريرا في ,

التاريخ : 19-09-2026 الوقت :

الطبيب المعالج : Reham Ahmed Fathy Ibrahim

يعتمد ,

Presidential Initiative for Women's Health - Pathology Lab
المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي

Patient Name : الأستاذه / سهام لطفى صبح ايوب
Patient No. : 28001073400068
Path No. : SR-496/22
Sex : Female
Age : 42 Y
Referreing Doc :

Request No. : 13007705
Request Date : 17/07/2022
Verification Date : 02/08/2022
Financial Category : اورام دار السلام - نفقة الدولة
Department : Out Patient
Registered by : Ali mostafa

Pathology Report

Clinical Data :
Right breast cancer.

Nature of the specimen:
Slide revision.

Gross Examination:
- One block labelled by (5030/22), one block labelled by (4576/22) and five slides received.

Microscopic examination:
- Histological type: Invasive duct carcinoma.
- Grade (differentiation+ nuclear pleomorphism + mitosis): GII (3+2+1).
- Lymphovascular invasion:Couldnt detecteed in received biopsy.
- perineural invasion No.
- Ductal carcinoma in situ: yes .
- Cytology of axilla : Positive for malignant cells.(metastasis).
- IPT:

Examination of ER stained slide show intense nuclear staining in 60% of the tumor cells.
Examination of PR stained slide show intense nuclear staining in 60% of the tumor cells.
Examination of HER2 stained slide show weak in complete membranous staining in<10% of tumor cells.
SISH on the referred slide: Not amplified.
* Average Her2 copy no: 3.05.
* Her2/CEP17 ratio: 1.7.
The repeated SISH on the tissue material at :Not amplified.
* Average Her2 copy no: 2.
* Her2/CEP17 ratio: 1.33.

Verified by :
DR.Menna Nabil

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name : الأستاذه / سهام لطفى صبح ايوب
Patient No. : 28001073400068
Path No. : SR-496/22
Sex : Female
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Request No. : 13007705
Request Date : 17/07/2022
Verification Date : 02/08/2022
Financial Category : اورام دار السلام - نفقة الدولة
Department : Out Patient
Registered by : Ali mostafa

Pathology Report

Diagnosis:

Right breast cancer; Slide revision:

- Invasive duct carcinoma GII with insitu component.
- No lymphovascular invasion.
- No perineural invasion.
- Cytology of axilla : Positive for malignant cells (metastasis).
- IPT:

ER: Positive (score 7/8).
PR: Positive (score 7/8).
HER-2: Equivocal (score 2+)
SISH : Not amplified on the referred slide and the repeated one.

Prof. Dr. Amal Hareedy.

Prof. Dr. Iman Gouda.

Dr. Menna Nabil.

Verified by :
DR.Menna Nabil

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name : الأستاذه / سهام لطفى صبح ايوب
Patient No. : 28001073400068
Path No. : 535/2023
Sex : Female
Age : 43 Y
Referreing Doc :

Request No. : 13004163
Request Date : 22/02/2023
Verification Date : 28/02/2023
Financial Category : اورام دار السلام - نفقة الدولة
Department : Out Patient
Registered by : .marwa mohamed youss

Pathology Report

CLINICAL DATA:
Right breast cancer, PSNA
NATURE OF SPECIMEN:
Right SM & RT SLN biopsy & RT extra-LNs
GROSS EXAMINATION:

- Procedure: Right simple mastectomy specimen measured 16x3x7cm with covering skin ellipse measured 14x7 cm .
- Tumor extent : An ill defined rubbery greyish white area
- Size: 5x3 cm .
- Skin involvement: unremarkable grossly
- Distance from margins: 2.5cm away from deep margin
- Lymph nodes number and size: Fibrofatty tissue piece measured 3x2 cm enclosing 5 LNs up to 2x1 cm .
- Frozen section: Free SLNs (0/4)

MICROSCOPIC EXAMINATION:

- Complete pathological response in previously diagnosed IDC
- Skin involvement: Free nipple and areola
- Lymph node involvement: Free LNs (0/9)
- Resection Margins: Free deep margin

DIAGNOSIS:
Right Simple Mastectomy & RT SLN & RT extra-LNs biopsy :

- Complete pathological response in previously diagnosed IDC
- Free deep margin
- Free SLNs and ALNs (0/9) with evidance of therapy induced fibrosis
- Stage : pT0, pN0

Prof. Dr. Iman Gouda
Dr.Hebatallah Mostafa

Verified by :
DR.Menna Nabil

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Name: سهام لطفي صبح ايوب
Date : 08/31/2022
Pat. ID: 10270631

WHOLE BODY ^{18}F-FDG PET/CT EXAMINATION

Clinical History

- A female patient 42 years old with history of right breast cancer.
- Pathologically proven invasive duct carcinoma
- For initial assessment .

Procedure:

- Using GE Discovery IQ simultaneous whole body 18F-FDG PET scan and multislice MSCT examination was performed from skull to lower thighs. Sixteen MSCT of the same level was performed for the purpose of attenuation correction, anatomical localization and diagnosis.
- Scan is done 1hour after IV contrast injection of 370MBq of 18 FDG.

Previous Exams:

❖ **No previous PET/CT study available for comparison.**

Findings:

I- BREAST AND AXILLARY REGIONS:

- **Low grade Metabolically active right breast parenchymal asymmetry densities / lesion is seen at right lower outer region; with SUVmax=2**
- **Few metabolically active right axillary LNs; the largest and most active is seen measuring 1.8cm in its maximum dimensions with SUVmax=2.6**
- No metabolically active left breast lesions.
- No metabolically active left axillary LNs.

PET Scan
[[DOCTYPE:PET_SCAN]]

Name: سهام لطفى صبح ايوب
Date : 08/31/2022
Pat. ID: 10270631

II- HEAD AND NECK:

- No metabolically active pharyngeal lesions.
- No metabolically active laryngeal lesions.
- Few subcentimetric bilateral upper deep cervical and submandibular LNs are seen showing no metabolic activity; likely non-specific.
- No metabolically active cervical lymphadenopathy.

III- CHEST:

- No metabolically active pulmonary nodules, masses, or cavitary lesions.
- No consolidation or collapse
- No metabolically active mediastinal lymphadenopathy.
- No pleural or pericardial effusion.

IV- ABDOMEN AND PELVIS

- Enlarged sized liver showing homogenous low parenchymal attenuation. No metabolically active hepatic focal lesions. No dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness. No stones or back pressure changes.
- Normal C.T appearance of the spleen, adrenal glands, pancreas, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or stones.
- No metabolically active pelvic lesions.
- No metabolically active abdominal or pelvic lymphadenopathy
- No ascites.

V- MUSCLOSKELETON:

- **Few multifocal sclerotic patchy metabolically inert osseous lesions are seen involving the spine and pelvic bones.**
- **No metabolically active lesions could be detected.**
- Spondylodegenrative changes of the spine.

PET Scan
[[DOCTYPE:PET_SCAN]]

Name: سهام لطفي صبح ايوب
Date : 08/31/2022
Pat. ID: 10270631

OPINION:

- ✓ Low grade metabolically active right lower outer breast parenchymal asymmetric densities / neoplastic lesion; for pathological correlation.
- ✓ Metabolically active right axillary metastatic lymphadenopathy
- ✓ Metabolically inert few sclerotic osseous lesions, for close follow up.
- ✓ PET –CT follow up is advisable.

Much Obliged;
Dr. Ahmed Osman, MD
Consultant Radiology

PET Scan
[[DOCTYPE:PET_SCAN]]

Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

A-Patient ID:
1- Name : سهام لطفي صبري أيوب
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2-ID number: 28001073400068
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Patient Age : 43 Y

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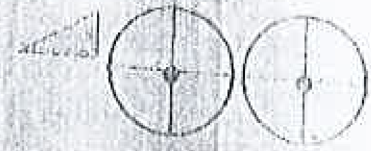

Family history
marital status
Co morbidity
Phone 01140027866
NBS Score
Menopausal status
GROUP B
REASON OF MDT POST OPERATIVE
Date of MDT 03/10/2023

Clinical data

Breast Imaging :

Mets Workup :

BX: IDC	ER: 7/8	PR: 7/8	Her 2 Neu : -VE	SISH :	KI67 :

T	N	M

Sono-mammography

PATIENT HISTORY

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CESM:COMPLETE RESPONSE
RT SIMPLE MASTECTOMY & RT SLNB RT EXTRA-LNS BIOPSY:
FROZEN SECTION--> FREE SLNS 0/4
COMPLETE PATHOLOGICAL RESPONSE IN PREVIOUSLY ID
FREE NIPPLE & AREOLA
FREE LN 0/9
FREE DEEP MARGIN
STAGE: PT0, PN0

MDT OUTCOME & PLAN

FOR PORT / ZOLADEX/TAMOXIFEN FOR 5 YEARS , shifted to AI / Zoladex
Zometa / 6month

SIGNATURE

MEDICAL ONCOLOGY		RADIOLOGY	
SURGERY		PATHOLOGY	
Psychiatric		Nutrition	

MDT Form
[[DOCTYPE:MDT_FORM]]

Presidential Initiative for Women's Health - Pathology Lab
المبادرة الرئاسية لصحة المرأة ـ معمل الباثولوجي

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DR.Menna Nabil

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[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name : الأستاذه / سهام لطفى صبح ايوب
Patient No. : 28001073400068
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Pathology Report
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المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي

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DIAGNOSIS:
Right Simple Mastectomy & RT SLN & RT extra-LNs biopsy :

- Complete pathological response in previously diagnosed IDC
- Free deep margin
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- Stage : pT0, pN0

Prof. Dr. Iman Gouda
Dr.Hebatallah Mostafa

Verified by :
DR.Menna Nabil

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Name: سهام لطفي صبح ايوب
Date : 08/31/2022
Pat. ID: 10270631

WHOLE BODY ^{18}F-FDG PET/CT EXAMINATION

Clinical History

- A female patient 42 years old with history of right breast cancer.
- Pathologically proven invasive duct carcinoma
- For initial assessment .

Procedu

- Usin covery IQ simultaneous whole body 18F-FDG PET scan and multislice
  M ation was performed from skull to lower thighs. Sixteen MSCT of
  18/03/2023 level was performed for the purpose of attenuation correction,
  atomical localization and diagnosis.
- Scan is done 1hour after IV contrast injection of 370MBq of 18 FDG.

Previous Exams:

- **No previous PET/CT study available for comparison.**

Findings:

I- BREAST AND AXILLARY REGIONS:

- **Low grade Metabolically active right breast parenchymal asymmetry densities / lesion is seen at right lower outer region; with SUVmax=2**
- **Few metabolically active right axillary LNs; the largest and most active is seen measuring 1.8cm in its maximum dimensions with SUVmax=2.6**
- No metabolically active left breast lesions.
- No metabolically active left axillary LNs.

PET Scan
[[DOCTYPE:PET_SCAN]]

Name: سهام لطفي صبح ايوب	
Date : 08/31/2022	
Pat. ID: 10270631	

II- HEAD AND NECK:

- No metabolically active pharyngeal lesions.
- No metabolically active laryngeal lesions.
- Few subcentimetric bilateral upper deep cervical and submandibular LNs are seen showing no metabolic activity; likely non-specific.
- No metabolically active cervical lymphadenopathy.

III- CHEST:

- No metabolically active pulmonary nodules, masses, or cavitary lesions.
- No consolidation or collapse
- No metabolically active mediastinal lymphadenopathy.
- No pleural or pericardial effusion.

IV- ABDOMEN AND PELVIS

- Enlarged sized liver showing homogenous low parenchymal attenuation. No metabolically active hepatic focal lesions. No dilated biliary radicles.
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V- MUSCLOSKELETON:

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- **No metabolically active lesions could be detected.**
- Spondylodegenrative changes of the spine.

PET Scan
[[DOCTYPE:PET_SCAN]]

Name: سهام لطفي صبيح ايوب
Date : 08/31/2022
Pat. ID: 10270631

OPINION:

- ✓ Low grade metabolically active right lower outer breast parenchymal asymmetric densities / neoplastic lesion; for pathological correlation.
- ✓ Metabolically active right axillary metastatic lymphadenopathy
- ✓ Metabolically inert few sclerotic osseous lesions, for close follow up.
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Much Obliged;
Dr. Ahmed Osman, MD
Consultant Radiology

PET Scan
[[DOCTYPE:PET_SCAN]]